

PAINFUL SKIN

Check if the patient needs urgent attention ➤ 67.

Red, warm, painful lump which may be fluctuant in the centre. May discharge pus.



© University of Cape Town

Boil/abscess likely

- If fluctuant, arrange incision and drainage.
- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days.
- If multiple lesions, lesion on face, extensive surrounding infection, temperature $\geq 38^{\circ}\text{C}$, HIV or diabetes, give antibiotic:
 - Give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days.
 - If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days.
- Advise to wash with soap and water, keep nails short and avoid sharing clothing or towels.
- If recurrent boils or abscesses:
 - Test for HIV ➤ 110 and diabetes ➤ 17.
 - Wash once with **chlorhexidine 0.05%** solution from neck down.
- Refer same day if:
 - BP < 90/60
 - Pulse > 100
 - Deep abscess difficult area to drain (hands, breast, perineum)
 - No response to treatment within 2 days

Red, warm, swollen skin
Are borders poorly or clearly defined?

Poorly-defined borders



© University of Cape Town

Cellulitis likely

Clearly-defined raised borders



© University of Cape Town

Erysipelas likely

- Give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days. If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days.
- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days.
- If limb affected, advise to keep elevated.
- Refer same day if:
 - BP < 90/60
 - Pulse > 100
 - Confused
 - Hand, face or scalp involvement
 - Extensive infection
 - Blisters or grey/black skin
 - Poorly controlled diabetes
 - Recurrent infections with underlying problem (like lymphoedema)
 - No response to treatment within 2 days

Painful blisters in a band along one side



© University of Cape Town

Herpes zoster (shingles) likely

- Test for HIV ➤ 110.
- Advise to keep lesions clean and dry, and to avoid skin contact with others until crusts have formed.
- Give **aciclovir** 800mg 5 times a day (4 hourly missing the middle of the night dose) for days if:
 - ≤ 3 days since onset of rash or
 - At risk of severe infection (> 65 years, HIV, diabetes, severe heart/liver disease or alcohol abuse) and lesions not yet crusted.
- For pain:
 - Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed.
 - If needed, add **tramadol** 50mg 6 hourly. If poor response, doctor to increase dose to **tramadol** 100mg 6 hourly.
 - If pain persists after rash has healed, give **amitriptyline**² 25mg at night. If no response, increase by 25mg every 2 weeks, up to 75mg if needed.
 - If still poor response, refer.
- If infected (skin red, warm, swollen):
 - Give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days.
 - If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days.
- Refer same day if:
 - Eye, ear or nose involvement
 - Suspected meningitis (headache, temperature $\geq 38^{\circ}\text{C}$, neck stiffness)
 - Rash involves more than one region

¹History of angioedema, anaphylaxis or urticaria. ²Avoid if on bedaquiline.